

VOID CALLING

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Summary

Void Calling, also known as the **Star Plague**, is an occult disease associated with long periods spent beyond a planetary atmosphere.

Settled Systems Code **A9%11&4-6** classifies it as **vacuum-associated cognitive exposure syndrome**, usually shortened to **vacuum sickness** in official reports. Spacers call it Void Calling because the afflicted rarely feel sick at first rather they hear someone outside the ship asking to be let in.

Early symptoms include broken sleep, voices beneath engine noise, movement outside viewports, and the persistent feeling that someone is standing just beyond the hull. As the disease advances, familiar people become harder to recognize, the interior of the ship begins to feel off, and open space appears safe, familiar, and occupied.

The lesser exposure syndrome can usually be treated before the victim answers the call. A person who enters vacuum while afflicted, encounters a **Void-Drawn**, or travels through certain regions of deep space can develop true Void Calling. In its final stage, the victim dies and rises as a creature able to survive in vacuum and spread the disease to others.

NATURE OF THE AFFLICTION

Void Calling is treated as a disease because it spreads between people, follows recognizable stages, responds to treatment, and can be contained through established shipboard procedures.

No consistent organism has ever been isolated from an afflicted person.

Transmission appears to occur through shared perception. Afflicted crew members often report related hallucinations without having discussed them. One hears knocking from outside the hull. Another sees a missing crewmate responsible for it. A third becomes convinced the airlock has already opened.

The disease does not appear during every long voyage. Recorded risk factors include:

- ◆ prolonged driftless travel outside a planetary atmosphere;
- ◆ repeated exposure to an unobstructed view of deep space;
- ◆ disrupted sleep and watch schedules;
- ◆ extended solitary duty;

- ◆ malfunctioning artificial horizons;
- ◆ unshielded occult or navigational systems;
- ◆ proximity to another afflicted person;
- ◆ contact with a Void-Drawn creature;
- ◆ travel through poorly charted frontier regions.

None of these conditions is present in every outbreak.

A natural planetary atmosphere interferes with the disease, even when that atmosphere is toxic or unbreathable. Artificial atmospheres aboard ships and stations do not provide the same protection.

VOID CALLING, EXPOSURE

DISEASE 6

DISEASE MENTAL OCCULT

Void Calling, Exposure is the lesser form of the disease found aboard ships, stations, and other habitats outside a planetary atmosphere. The victim begins to perceive familiar people and ordinary surroundings as incomplete or unreal. Voices and apparent emergencies draw their attention toward viewports, maintenance passages, and airlocks.

Exposure A creature can be exposed after spending at least 24 consecutive hours outside a planetary atmosphere aboard a vessel or habitat without a functioning void-grounding protocol. A creature is also exposed after spending 8 consecutive hours aboard the same vessel or habitat as a creature at stage 3 or higher, or through direct contact with a Void-Drawn creature.

A creature with the cosmic trait can't progress beyond stage 1.

While outside a planetary atmosphere, this disease gains the virulent trait whenever the afflicted creature attempts a stage save within 30 feet of another creature afflicted with either form of Void Calling.

While within a natural planetary atmosphere, this disease loses the virulent trait. When you succeed at a stage save while within such an atmosphere, reduce the disease's stage by 2. On a critical success, reduce its stage by 3.

Saving Throw DC 22 Will

Onset 3 days

Stage 1 carrier with no effects (1 day)

Stage 2 stupefied 1; you intermittently hear a familiar voice speaking through ventilation, machinery noise, or the exterior hull (4 hours)

Stage 3 stupefied 1; you can't willingly move within 10 feet of another creature unless you succeed at a DC 5 flat check. Attempt this check once for each move action that would bring you within 10 feet of another creature. On a failure, the action is lost. (4 hours)

Stage 4 as stage 3, and all other creatures are hidden from you. A creature is observed by you while it is within your reach. When a creature within 30 feet uses a move or manipulate action, it becomes observed by you until the end of that creature's turn. You can't use Seek to make a creature observed beyond these circumstances. (1 day)

Stage 5 as stage 4. At the start of each of your turns in an encounter, or once every 10 minutes during exploration, attempt a Will save against the disease's DC.

On a failure, you must spend your first action moving toward the nearest known airlock, exterior hatch, maintenance lock, or other route into open space. If you are adjacent to such an access point, you must instead use your first action to Interact with it.

On a critical failure, you must spend all available actions doing so.

If no route is known to you, you follow a direction indicated by your hallucinations. This is a compulsion effect.

If you enter vacuum while at stage 5, you are immediately exposed to true Void Calling. Void Calling, Exposure then ends. (1 hour)

VOID CALLING

DISEASE 10

DISEASE MENTAL OCCULT VIRULENT

True Void Calling begins when the victim answers the lesser disease, encounters a strong manifestation of the Calling, or is exposed to a Void-Drawn creature. Open space no longer appears empty. The afflicted sees people, lights, structures, landscapes, and paths that no unaffected observer can perceive.

A creature with the cosmic trait can't progress beyond stage 1.

While within a natural planetary atmosphere, Void Calling loses the virulent trait. When you succeed at a stage save while within such an atmosphere, reduce the disease's stage by 2. On a critical success, reduce its stage by 3.

Exposure Entering vacuum while at stage 5 of Void Calling, Exposure; spending 10 minutes within 30 feet of a Void-Drawn creature; or direct exposure to a powerful source of the Calling, as determined by the GM.

Saving Throw DC 27 Will

Onset 1 hour. A creature exposed by entering vacuum while at stage 5 of Void Calling, Exposure has no onset and immediately advances to the appropriate stage after its initial save.

Stage 1 stupefied 1. You don't need to breathe and can survive the basic environmental effects of vacuum. You still suffer from cold, radiation, and other hazards not caused solely by vacuum. (1 day)

Stage 2 stupefied 2 and drained 1. You gain resistance 5 to cold and void damage. Voices outside the vessel address you by names, titles, or relationships known only to you. (4 hours)

Stage 3 as stage 2, and all other creatures are hidden from you unless they are within your reach. When a creature within 30 feet uses a move or manipulate action, it becomes observed by you until the end of that creature's turn. You can't use Seek to make a creature observed beyond these circumstances. (4 hours)

Stage 4 as stage 3. At the start of each of your turns in an encounter, or once every 10 minutes during exploration, attempt a Will save against the disease's DC.

On a failure, you must spend your first action moving toward open space or operating a device that would provide access to it. On a critical failure, you must spend all available actions doing so.

The first time each round a creature physically prevents you from following this compulsion, you must attempt another Will save against the disease's DC. On a failure, you become confused for 1 round. This is a compulsion effect. (4 hours)

Stage 5 stupefied 3 and drained 2. Your resistance to cold and void damage increases to 10. You can communicate only through short phrases spoken by figures in your hallucinations.

At the start of each of your turns, you must attempt the compulsion save described in stage 4. On any failure, you must spend all available actions following the call. Whenever another creature prevents you from doing so, you become confused for 1 round. (1 hour)

Stage 6 death. After 1d4 rounds, your body rises as a Void-Drawn creature under the GM's control.

HALLUCINATIONS

Void Calling draws from the victim's memories, fears, expectations, and unfinished obligations. The result is personal enough to sound convincing.

Common hallucinations include:

- ◆ a missing family member knocking on the exterior hull;
- ◆ a child or companion drifting beyond a viewport without a suit;
- ◆ an emergency beacon broadcasting the victim's own voice;
- ◆ a maintenance worker signaling from an impossible exterior platform;
- ◆ a familiar settlement visible between the stars;
- ◆ a dead friend insisting that the vessel has already depressurized;
- ◆ crewmates appearing hollow, artificial, or slightly out of synchronization;
- ◆ stars that move when viewed indirectly;
- ◆ a corridor that seems to continue through a sealed exterior door;

- ◆ the certainty that everyone else has already evacuated;
- ◆ the belief that the ship is a replica built around the victim;
- ◆ someone outside asking the victim to open the door quietly.

The first hallucinations are rarely threatening. They ask for help, offer reassurance, or provide a plausible reason to leave the inhabited compartment.

RECOGNITION AND DIAGNOSIS

Early Void Calling can be mistaken for sleep deprivation, isolation stress, decompression injury, ordinary fear, hostile telepathy, navigational sickness, or sabotage.

Medicine can identify the recurring symptom pattern. Occultism can recognize the hallucinations as a coordinated influence rather than an ordinary sensory disturbance.

Physical examinations often reveal:

- ◆ disrupted sleep without corresponding stimulant use;
- ◆ repeated attention toward exterior access routes;
- ◆ difficulty recognizing familiar faces;
- ◆ contradictory reports of movement outside the hull;
- ◆ related hallucinations among people who have not spoken;
- ◆ normal pressure readings during reported decompression;
- ◆ attempts to disable viewport filters or airlock interlocks;
- ◆ unexplained resistance to vacuum during true Void Calling.

No single symptom confirms the disease.

Ship crews are expected to investigate reports of exterior distress until independent sensors rule them out. Some crews have died after dismissing real emergencies as hallucinations. Others have opened airlocks in response to emergencies that never occurred.

PREVENTION

Long-voyage precautions usually include:

- ◆ paired or rotating watch assignments;
- ◆ limits on solitary duty;
- ◆ artificial horizon displays;
- ◆ scheduled changes in lighting, temperature, scent, and ambient sound;
- ◆ regular spoken identity and location checks;

- ◆ physical exercise under artificial gravity where available;
- ◆ scheduled periods with viewports covered;
- ◆ independent confirmation of exterior alarms;
- ◆ two-person authorization for airlock operation;
- ◆ mechanical locks that function during software failure;
- ◆ daily review of sleep, memory, and sensory disturbances;
- ◆ immediate removal from piloting, weapons, and airlock duty after credible symptoms;
- ◆ plans for diversion toward a planetary atmosphere if symptoms worsen.

Crew members are encouraged to choose **anchors** before departure. An anchor can be a person, object, recording, smell, song, routine, or shared memory that confirms where the person is and why they intend to remain there.

Specific anchors work better than generic reassurance. A sibling describing an embarrassing childhood injury, a tool with a familiar damaged grip, or a meal prepared from a family recipe can interrupt the hallucination long enough for the victim to recognize it.

TREATMENT

Ordinary Treat Disease actions can assist recovery.

Medicine addresses exhaustion, malnutrition, injury, and the physical damage caused during an episode. Occult treatment helps the victim separate personal memory from the version of that memory used by the Calling.

Solitary confinement is considered dangerous. Isolation removes contradictory sensory information and gives the disease more control over the victim's perceived surroundings.

Treatment areas usually include:

- ◆ at least two consistent caregivers;
- ◆ no direct access to exterior locks;
- ◆ covered or filtered viewports;
- ◆ familiar physical objects;
- ◆ stable lighting and gravity;
- ◆ repeated orientation to current stamp, location, and personnel;
- ◆ manual environmental controls outside the patient's reach;
- ◆ independent verification of all reported emergencies.

Returning the victim to a natural planetary atmosphere is the most reliable nonmagical treatment. The atmosphere does not need to be breathable. A sealed habitat inside a toxic atmosphere still provides the beneficial effect.

Effects that counteract or remove diseases can cure either form normally. Recovery does not erase memories of the hallucinations.

SETTLED SYSTEMS CLASSIFICATION

Settled Systems Code **A9%11&4-6**, the **Extended-Voyage Cognitive Exposure and Quarantine Standard**, establishes the official term *vacuum-associated cognitive exposure syndrome* and sets minimum response procedures.

Registered long-voyage vessels must maintain:

- ◆ documented watch rotations;
- ◆ an approved grounding protocol;
- ◆ restricted airlock access;
- ◆ procedures for removing symptomatic personnel from flight-critical duty;
- ◆ records of unexplained exterior alarms;
- ◆ access to treatment or diversion toward an atmosphere;
- ◆ a formal report after any stage-3 or greater incident.

The law exists because one afflicted pilot, engineer, or security officer can kill an entire crew.

Its paperwork also gives operators a way to limit liability. A company with complete grounding logs and signed crew acknowledgments can argue that an outbreak resulted from individual noncompliance. Ships with incomplete records may lose insurance coverage, docking eligibility, or survivor compensation even when the required equipment failed.

Corporate security often interprets quarantine authority as permission to sedate and isolate afflicted workers. The actual protocol requires supervision and sensory grounding. Solitary restraint remains common aboard ships without medical personnel or spare compartments.

DISCOVERY

The first recorded outbreak is difficult to identify.

The earliest human and dwarven crews traveling after the **Star Dream** reported nothing similar. Their routes remained close to inhabited systems, their voyages were comparatively short, and their crews maintained constant contact while learning unfamiliar technology.

The first surviving accounts appear after expeditions moved beyond the region now recognized as the **Settled Systems**. Separate crews reported the same details:

- ◆ knocking from outside the hull;
- ◆ relatives visible between stars;
- ◆ crew members walking willingly into maintenance locks;
- ◆ exterior suits found empty and intact;
- ◆ survivors insisting that missing crew remained visible;
- ◆ recovered bodies moving without atmosphere.

Several expeditions returned before their symptoms became obvious. Later outbreaks occurred aboard vessels that had never traveled as far into the frontier.

By Lapse 2, long driftless voyages commonly used paired watches, mechanical airlock restrictions, covered viewport periods, and spoken grounding checks. Many of those procedures survive as spacer customs even when the crews following them no longer know where they came from.

Experienced spacers still refuse to answer unexplained knocking on an exterior hull.

COMPETING EXPLANATIONS

Medical authorities usually describe Void Calling as an occultly transmissible neuroperceptual disease activated by prolonged environmental stress.

Star Dream theologians have proposed several explanations:

- ◆ a corrupted fragment of the original revelation;
- ◆ something that heard the Star Dream and began answering;
- ◆ punishment for entering regions the Elect were never shown;
- ◆ another population attempting contact through incompatible senses;
- ◆ evidence that open space possesses some form of awareness.

Moyishuu traditions generally reject the claim that the disease comes from the Drift. Their inherited dreams distinguish the structured movement of the Drift from the stillness associated with the Calling.

No explanation accounts for every outbreak.

PLAYER OPTIONS

GROUNDING CHECK

AUDITORY CONCENTRATE MENTAL

ACTIVITY 10 MINUTES

You guide an afflicted creature through familiar memories, physical sensations, current surroundings, and independently verified facts.

Requirements You are trained in Diplomacy, Medicine, or Occultism; the target is willing, within your reach, and able to perceive you.

Attempt a Diplomacy, Medicine, or Occultism check against the DC of the target's Void Calling affliction. The target is temporarily immune to further Grounding Checks for 8 hours.

Critical Success The target gains a +2 circumstance bonus to its next stage save against Void Calling. For 10 minutes, the target treats creatures within 30 feet as observed unless another effect would make them hidden or undetected.

Success The target gains a +1 circumstance bonus to its next stage save against Void Calling.

Failure You provide comfort but no mechanical benefit.

Critical Failure As failure.

A bonus from Grounding Check doesn't stack with the bonus from Treat Disease.

VOIDWISE

FEAT 1

GENERAL SKILL

Prerequisites trained in Occultism

You have studied the symptoms, spacer traditions, and containment procedures associated with Void Calling.

You can use Occultism in place of Medicine to Treat Disease when the target is afflicted with either form of Void Calling.

While you can perceive at least one allied creature within 30 feet, you gain a +1 circumstance bonus to saving throws against Void Calling.